

Pathogen Cards + Case Drills + Tables 30.1–30.3

Built from the two missed exam questions — drills the exact properties, pattern recognition, and Maciocia comparison tables tested on the quiz.

Why This Exists

The "**All of the following are true about Cold EXCEPT**" question tests property knowledge, not just clinical signs. Cold *congeals, contracts, produces clear discharge* — it does NOT cause bleeding. Heat causes bleeding. Dampness causes turbid discharge. Wind causes migratory symptoms. These are drilled here.

Section 1 — 6 Pathogen Cards (2–3 per page)

Section 2 — 12 Cases then Answers

Section 3 — Maciocia 30.1 · 30.2 · 30.3

Final — Rapid-Fire Master Cheat Sheet



Section 1

Six Exogenous Pathogen Properties

2–3 cards per page · Properties for EXCEPT-style questions

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Cold (Hán) — Yīn Pathogen · Winter

Injures Yáng Qì · Areas: Exterior (short) or Interior (long)

- 1Belongs to Yīn · Injures Yáng Qì Cold limbs, pallor, preference for warmth.
- 2Causes Contraction (收引) Contracts pores → NO sweating; contracts tendons/vessels → stiff neck, tight muscles, restricted movement. Floating-Tight pulse.
- 3Causes Stagnation (凝滯) Impairs free flow of Qì and Blood → pain WORSE with cold, BETTER with warmth. Deep-Tight or Deep-Slow pulse.
- 4Congeals Blood Slows and thickens Blood → fixed stabbing pain, cold extremities. NOT the same as bleeding.
- 5Produces CLEAR, Watery Discharge All secretions clear: clear nasal mucus, profuse clear urine, loose watery stools, thin clear phlegm. Tongue: pale, white moist coat.

EXCEPT Trap

✓ Cold DOES: contract · stagnate · congeal blood · clear discharge · injure Yáng
X Cold does NOT cause BLEEDING — that is Heat/Fire only

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Heat/Fire (Rè/Huǒ) — Yáng Pathogen · All Seasons

3 internal sources (external · emotional/diet · transformation of Cold) · Most severe = Fire

- 1Burning & Upward — Yáng Rises to head/face: headache, red face, red eyes, agitation.
- 2CAUSES BLEEDING (key differentiator from Cold) Speeds and forces Blood out of vessels → epistaxis, haematuria, haemoptysis, heavy periods, blood in stools.
- 3Consumes Yīn Fluids Strong thirst (wants cold, large amounts); dry mouth; scanty dark urine; constipation.
- 4Causes Redness + Thick/Yellow Discharge Red face/tongue/eyes; yellow nasal mucus; dark urine; dry hard stools; thick yellow phlegm; yellow coat.
- 5Disturbs Shén + Stirs Wind Irritability, restlessness, mania (Shén). Extreme Heat → Liver Wind → fever + convulsions + opisthotonos.

Cold vs Heat — The Core Contrast

Cold: CLEAR discharge + NO bleeding + Pain BETTER with warmth + Pale tongue
Heat: YELLOW/RED discharge + BLEEDING + Pain WORSE with heat + Red tongue

Exterior Cold vs Exterior Heat — Side by Side

Sign	Wind-Cold (Exterior Cold)	Wind-Heat (Exterior Heat)
Sweating	NO sweating (Cold contracts pores)	Slight sweating (Heat opens pores)
Thirst	NO thirst	YES thirst
Throat	No sore throat	Sore throat ✓
Pulse	Floating-Tight	Floating-Rapid

The EXCEPT rule: Cold does NOT cause BLEEDING

Cold DOES: ① Contract (no sweat) ② Stagnate (pain worse cold) ③ Congeal blood (not bleeding!) ④ Produce **clear** discharge ⑤ Injure Yáng
Heat DOES: ① Cause **BLEEDING** ② Produce **yellow/red thick** discharge ③ Cause redness ④ Consume Yīn ⑤ Disturb Shén ⑥ Stir Wind



Dampness (Shī) — Yīn Pathogen · Late Summer

Spleen "hates Damp" → Prolonged/intractable diseases · Sticky/greasy coat is definitive

- 1
- Heavy & Turbid
- Body heavy "like lead"; head wrapped in cloth; limbs heavy. ALL discharges TURBID/MURKY (not clear like Cold, not bloody like Heat): cloudy urine, weeping eczema, purulent leucorrhoea.
- 2
- Viscous & Stagnant — Prolonged Disease
- Sticky/greasy tongue coat. Diseases intractable: eczema, intestinal typhoid, fungal infections, Damp-Bi don't resolve quickly.
- 3
- Impairs Yáng (esp. Spleen Yáng)
- Obstructs Qì circulation → fullness in chest, epigastric distension, poor appetite, nausea. Loose stools, difficult urination.
- 4
- Settles Downward
- Oedema in ankles/legs first, turbid vaginal discharge, diarrhea, blood in urine — lower body affected first.

Damp vs Cold vs Heat

Damp: TURBID discharge (not clear, not bloody) · STICKY/GREASY coat · HEAVY sensation

✗ Does NOT cause bleeding (Heat) · Does NOT cause contraction (Cold) · Pulse: Slippery or Soggy



Wind (Fēng) — Yáng Pathogen · Spring · Primary Pathogen

"Leader of all pathogens" → all others need Wind to invade. Floating pulse always

- 1
- Primary/Leader Pathogen — Always Upward & Outward
- Wind-Cold, Wind-Heat, Wind-Damp all require Wind. Invades head, face, skin, muscles first: headache, stiff neck, nasal obstruction.
- 2
- MIGRATORY — Wandering Symptoms
- Symptoms MOVE and are NOT fixed: wandering joint pain (Wind Bi); urticaria (wheals appear/disappear); moving pain. "If it moves, think Wind."
- 3
- Rapid Changes + Tremor/Convulsion
- Abrupt onset; rapid changes; short duration. Causes movement: dizziness, tremors, convulsions, opisthotonos, deviation of mouth/eyes (Wind-Phlegm).
- 4
- Opens Pores → Sweating + Aversion to Wind
- Pores opened by Wind (vs Cold which CLOSES pores → no sweating). Aversion to drafts/wind is hallmark. Floating pulse.

Wind Memory: MOVING = Wind. Fixed = Damp or Cold.

Wandering joint pain, urticaria appearing/disappearing, tremors, dizziness = Wind. Anything stationary/fixed = not Wind.

Pathogen Matrix — Which One? (Exam-ready comparison)

Property	Cold	Heat	Damp	Wind
Discharge	CLEAR	YELLOW/RED thick	TURBID/murky	Variable
Bleeding	✗ NO	✓ YES	✗ NO	✗ NO
Pain quality	Worse cold; fixed	Burning	Heavy; fixed	MIGRATORY
Coat	White moist	Yellow dry	STICKY	Thin white
Unique	Contracts pores (no sweat)	Redness	Head in cloth	MIGRATORY

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Dryness (Zào) — Autumn · Attacks Lung ("Delicate Zang")

Two types: Cool Dryness (early Autumn) vs Warm Dryness (late Autumn/warm climate)

- 1

Consumes Fluids — DRYNESS Everywhere Dry nose, dry throat, dry mouth with thirst, chapped lips, wrinkled skin, dry cough with scanty/sticky sputum, constipation. If all orifices are dry → Dryness or Yīn def.
- 2

Primarily Attacks the Lung Impairs Lung's dispersing/descending → dry cough, scanty sticky/bloody sputum, hoarseness. Invades via nose and throat.
- 3

Cool Dryness vs Warm Dryness **Cool:** aversion to cold, no sweat, dry nasal passages, dry cough, headache — Floating-Tight. **Warm:** fever, headache, thirst, dry cough — Floating-Rapid. Both: dry tongue, little coat.

Dryness vs Dampness vs Heat

Dryness: fluids DEPLETED (dry everywhere, no discharge). Heat: fluids forced OUT as yellow/thick/bloody discharge. Damp: fluids EXCESS but turbid/obstructed.

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Summer Heat (Shǔ) — THE UNIQUE PATHOGEN

SUMMER ONLY · No endogenous form · Always external · Always combined with Damp in clinic

- 1

ONLY occurs in Summer — NO Endogenous Form THE defining characteristic. All other pathogens have internal counterparts. Summer Heat does NOT. "Internal Summer Heat" in an exam = wrong.
- 2

Extreme Yáng — Concurrent Qì Deficiency Profuse sweating carries Qì out → Qì def concurrent with Heat. High fever + surging pulse + lassitude + reluctance to speak — all AT SAME TIME.
- 3

Almost Always + Dampness (Summer = humid) Summer Heat + Damp: fever + restlessness + thirst (Heat) PLUS heaviness + nausea + chest oppression + poor appetite (Damp).

Exam Trap: No Internal Summer Heat

✓ Summer Heat collapses patient outdoors in July heat with fever + surging pulse + concurrent Qì def fatigue

X Never select "internal Summer Heat" — there is no endogenous form. If in summer + outdoor + collapse = Summer Heat.

Quick Drill — Pick the Correct Pathogen

1. Dry skin, dry cough, dry nose, NO discharge → DRYNESS

2. Turbid urine, greasy coat, heavy body, prolonged disease → DAMP

3. Clear nasal discharge, no sweating, no thirst → COLD

4. Epistaxis (bloody nose) + high fever + rapid pulse → HEAT

5. Wandering joint pain that MOVES from joint to joint → WIND

6. July, outdoor worker, sudden collapse, concurrent fatigue → SUMMER HEAT

7. Which pathogen has NO endogenous (internal) form? → SUMMER HEAT only

8. Which pathogen CONGEALS blood but does NOT cause bleeding? → COLD

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Section 2

Interior vs Exterior — 12 Clinical Cases

All cases first · Answers follow · Cover answers while reading cases

Key Rule: Exterior = BOTH fever AND aversion to cold TOGETHER (neither alone). Interior = one WITHOUT the other, OR neither. Floating pulse = Exterior. Deep pulse = Interior. Thin white coat = Exterior. Yellow/black/greasy coat = Interior.

Case 1

28F — sudden onset

Fever, pronounced chills, NO sweating, severe body aches, stiff neck. Tongue: thin white coat. Pulse: Floating-Tight.

→ Exterior or Interior? What pattern?

Case 2

35M — 1 day onset

Fever, MILD aversion to cold, slight sweating, sore throat, thirst. Tongue: thin white coat, slightly red on sides/front. Pulse: Floating-Rapid.

→ Exterior or Interior? Cold or Heat?

Case 3

50M — 3 days

HIGH fever, NO aversion to cold, strong thirst for cold drinks, constipation, dark scanty urine, red face, very restless. Tongue: red with yellow coat. Pulse: Full-Rapid.

→ Exterior or Interior? Full or Empty?

Case 4

62F — chronic, years

Cold limbs, preference for warmth, NO fever, fatigue, pale face, loose stools, frequent urination. Tongue: pale, thin white coat. Pulse: Deep-Slow-Weak.

→ Exterior or Interior? Full Cold or Empty Cold?

Case 5

41F — 2 days, combined

Fever + aversion to cold + no sweating + headache + body aches (Exterior signs) PLUS irritability, thirst, desire for cold drinks.

→ Single pattern or combined? What is it?

Case 6

47M — chronic evening symptoms

Low-grade fever AFTERNOON/EVENING only. Malar flush (cheekbones only). Night sweats. Heat in 5 palms (hands/feet/chest). Dry mouth at night — sips only. No aversion to cold. Tongue: red, PEELED. Pulse: Floating-Empty-Rapid.

→ Exterior or Interior? Full Heat or Empty Heat?

Tricky territory: Mixed patterns, False Heat/Cold, disease transmission, Full vs Empty. Use TONGUE as your anchor when the clinical picture seems contradictory — the tongue doesn't lie.

Case 7

68F — critically ill

Red cheeks, slight fever, slight irritability — BUT: body is actually cold, patient curled up wanting warmth, pale WET tongue, quiet weak breathing, prefers warm drinks. Pulse: Rapid, Floating, Big but EMPTY (hollow feeling).

→ Is this real Heat? Or something else? What pattern?

Case 8

55M — ICU

Cold limbs — BUT: chest and abdomen are HOT underneath. High fever. Loud voice. Very irritable. Constipation. Scanty dark urine with burning sensation. Dark face. RED, DRY tongue. Pulse: Deep, FULL.

→ Is the cold limbs sign reliable here? What is this?

Case 9

38M — acute

Slight fever with mild aversion to cold for 2 days (Exterior). NOW: aversion to cold has COMPLETELY DISAPPEARED. Fever persists. New: irritability + thirst + yellow tongue coat. Pulse changed from Floating to Deep.

→ What happened? What stage is this now?

Case 10

58M — acute onset

Severe abdominal pain, WORSE with pressure, WORSE with heat, loud strong voice, constipation, scanty dark urine, red face. Tongue: red with yellow coat. Pulse: Full-Rapid.

→ Full or Empty? Hot or Cold? Exterior or Interior?

Case 11

65F — chronic, years

Dull abdominal pain, BETTER with pressure, BETTER with warmth, quiet voice, preference for lying curled up, fatigue, loose stools, pale face. Tongue: pale, thin white coat. Pulse: Deep-Slow-Weak.

→ Full or Empty? Hot or Cold?

Case 12

45M — chronic

Low-pitched tinnitus that IMPROVES with rest and gentle pressure. Dizziness, fatigue, chronic lower back weakness. Condition has lasted years. Pulse: Deep-Fine-Weak. Tongue: Pale.

→ Full or Empty? Which organ? Tinnitus pitch is a clue.

Case 1 ✓

Exterior — Wind-Cold (Full)

BOTH fever AND chills together = Exterior. No sweating = Cold (pores contracted). Floating-Tight pulse = Exterior Cold. Thin white coat = Exterior. Severe aches + stiff neck = Wind-Cold invasion.

Logic: Fever + chills = Exterior. Tight pulse = Cold. Full = pathogen present and strong (severe pain, no deficiency signs). Floating = still on Exterior.

Case 2 ✓

Exterior — Wind-Heat

BOTH fever AND chills = Exterior. Slight sweating + thirst + sore throat = Heat (Heat opens pores slightly). Floating-Rapid pulse. Red tongue on sides/front = beginning of Heat penetrating.

Key differentiator from Wind-Cold: Slight sweating (Heat opens pores, Cold closes them), thirst, sore throat. Rapid vs Tight pulse.

Case 3 ✓

Interior — Full Heat (Interior Excess Yáng)

Fever WITHOUT aversion to cold = Interior. Full-Rapid pulse + thick yellow coat + constipation + dark urine = Interior Full Heat. Strong/intense symptoms = Full (not Empty).

Key: No aversion to cold = Interior. Everything intense = Full. Yellow coat + red tongue = Heat. Full-Rapid pulse confirms Full Heat.

Case 4 ✓

Interior — Empty Cold (Yáng Deficiency)

Aversion to cold WITHOUT fever = Interior. Chronic = Interior. Deep-Slow-Weak pulse = Interior Empty Cold. Pale tongue = Yáng def. All signs mild = Empty (not Full).

Key: Chronic = Interior. Deep-Slow-Weak = Interior Empty Cold. Pale tongue + thin white coat (not thick) = Yáng def, not Cold pathogen excess.

Case 5 ✓

Combined — Exterior Cold + Interior Heat

Exterior signs (fever + chills + no sweat + aches) coexist with Interior Heat signs (irritability + thirst + cold drinks). Both patterns are simultaneously present.

Key: Neither excludes the other — they coexist. Interior Heat was pre-existing when Exterior Cold invaded. Treatment: resolve Exterior first, then address Interior Heat.

Case 6 ✓

Interior — Empty Heat (Yīn Deficiency)

No aversion to cold = Interior. Malar flush ONLY (not whole face), afternoon/evening timing, 5-palm heat, peeled tongue = CLASSIC Empty Heat. Floating-Empty-Rapid pulse = Yīn deficiency. DIFFERENT from Full Heat.

Key differentiator from Full Heat: Partial flush (malar only), partial timing (afternoon), small sips at night, PEELED tongue (no coat). Full Heat = whole face red all day, strong thirst, yellow coat.

Case 7 ✓

True Cold — False Heat (真寒假熱)

The red cheeks and slight fever are FALSE. True signs: cold body curled up, pale WET tongue, Minute-Deep pulse, prefers warmth. There is NO actual Heat — extreme Cold is forcing Yáng upward as a last refuge (Floating Yáng).

Key: Pale WET tongue = Cold (not Heat). Check tongue when face color confuses you. Rapid/Big pulse that feels EMPTY (hollow) = False Heat, not Full Heat.

Case 8 ✓

True Heat — False Cold (真熱假寒)

Cold limbs are FALSE. True signs: hot chest/abdomen, red DRY tongue, Full-Deep pulse, loud voice, constipation, dark urine. TRUE Heat hidden inside with FALSE Cold on the exterior (limbs).

Key: Red DRY tongue = Heat. Full-Deep pulse = Interior Full. Cold limbs are just Heat blocking circulation to extremities. Same principle as "heat above, cold below."

Case 9 ✓

Disease Transmission — Exterior → Interior (Inward, Worsening)

Aversion to cold DISAPPEARED + fever persists + irritability + thirst + yellow coat + pulse changed Floating → Deep = pathogen has penetrated from Exterior to Interior. Disease is progressing inward (bad sign).

Key rule: When aversion to cold disappears and aversion to heat begins, the disease has moved Interior. Pulse going from Floating to Deep confirms this.

Case 10 ✓

Interior — Full Heat (Excess)

Pain WORSE with pressure = Full. Severe/intense = Full. Loud voice = Full. Interior confirmed by Full-Rapid pulse + yellow coat + constipation. Heat confirmed by red face, dark urine, red tongue/yellow coat.

Full vs Empty pain rule: WORSE with pressure = Full. BETTER with pressure = Empty. Combined with intense onset + strong constitution = Full pattern.

Case 11 ✓

Interior — Empty Cold (Yáng Deficiency)

Pain BETTER with pressure + warmth = Empty Cold. Quiet voice = Empty. Chronic + Deep-Slow-Weak pulse = Interior Empty Cold (Yáng def). Pale tongue confirms deficiency of Yang/Blood.

Mirrors Case 4 (same pattern): Chronic + pale tongue + better with warmth/pressure + Deep-Slow-Weak = Empty Cold. Treat: tonify Yáng (reinforcing method).

Case 12 ✓

Empty Condition — Kidney Deficiency

Low-pitched tinnitus IMPROVING with rest = Empty. Chronic + Deep-Fine-Weak pulse + lower back weakness = Kidney deficiency (Kidney opens to ear). Pale tongue = Blood or Yáng deficiency.

Tinnitus rule: High-pitched + WORSE with pressure = Full (Liver Fire). Low-pitched + BETTER with rest = Empty (Kidney def). Chronic = always more likely Empty.

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Section 3

Maciocia Tables 30.1 · 30.2 · 30.3

Full/Empty Heat · Full/Empty Cold · False Heat/Cold

Mechanism: Full-Heat = external or internal Yáng pathogen (excess). Empty-Heat = Yīn depleted → apparent excess of Yáng (deficiency). Both have fever but the clinical picture is very different.

Sign	Full-Heat 實熱 (Excess Yáng)	Empty-Heat 虛熱 (Yīn Def)
Face	Whole face RED — all day constant	Malar flush ONLY (cheekbones) — afternoon/eve only
Thirst	Strong thirst, wants cold water (large amounts)	Dry mouth at night; wants only small sips
Eyelid	Red all over inside eyelid	Thin red line inside eyelid only
Taste	Bitter taste; mouth ulcers	No bitter taste; dry throat at night
Timing	ALL DAY (constant, high fever)	AFTERNOON or EVENING only; low-grade
Hallmarks	Thirst + restlessness + red face + dry stools + dark urine + profuse bleeding	Night sweats · Heat in 5 palms · Insomnia · Slight bleeding only
Sleep	Dream-disturbed, very restless	Waking frequently at night/early morning
Skin	Red, hot, PAINFUL skin eruptions	Scarlet but not raised; NOT painful
Pulse	Full-Rapid-Overflowing	Floating-Empty-Rapid (hollow under finger)
Tongue	Red with YELLOW coating	Red and PEELED (no coating at all)
Treatment	Clear Heat (reducing method, cold herbs)	Nourish Yīn; clear Empty-Heat (shi gao, mai dong)

Hook: Full Heat = EVERYTHING intense. Empty Heat = PARTIAL everything.
Partial flush (malar only) · Partial timing (afternoon) · Partial coat (peeled) · Partial thirst (small sips at night). Floating-Empty-Rapid pulse is pathognomonic.

Table 30.2: Full-Cold (Excess Yīn) vs Empty-Cold (Yáng Def) · Table 30.3: True Cold–False Heat vs True Heat–False Cold

Sign	Full-Cold 實寒 (Excess Yīn)	Empty-Cold 虛寒 (Yáng Def)
Face	Bright-white, almost shiny	Dull-white, sallow
Pain	Sharp, intense — WORSE with pressure	Dull ache — BETTER with pressure + warmth
Bowels	BETTER after bowel movement	WORSE after bowel movement
Onset	Sudden, acute	Gradual, chronic
Pulse	Full-Tight-Deep	Weak-Slow-Deep
Tongue	THICK white coating	THIN white coating
Cause	External Cold penetrating; cold foods; direct invasion of ST/Intestines/Uterus	Overwork; cold diet; excessive sex; Kidney-Yáng def; prolonged Full-Cold

Table 30.2 Hook: **PRESSURE is the key.** Full Cold: **WORSE** with pressure (Full rule). Empty Cold: **BETTER** with pressure and warmth (Empty rule). Thick coat = Full; Thin coat = Empty.

Sign	True Cold–False Heat (真寒假熱)	True Heat–False Cold (真熱假寒)
Appearance	Red cheeks (like powder, not true red); slight irritability	Cold limbs (but chest/abdomen is hot)
True signs	Body cold, curled up; pale WET tongue; Minute-Deep pulse; prefers warmth; low voice	Red DRY tongue; Full-Deep pulse; loud voice; hot chest; constipation; dark urine
Pulse	Rapid, Floating, Big but EMPTY (hollow)	FULL and Deep (cold limbs are deceptive)
Tongue	Pale, wet (confirms True Cold)	Red, dry (confirms True Heat)
Urine	Clear and profuse	Scanty and dark
Treatment	WARM — treat True Cold	CLEAR HEAT — treat True Heat

False ≠ Empty. Critical distinction:

Empty-Heat: Yīn deficient → REAL relative Heat exists (night sweats, 5-palm heat = REAL Heat signs)
False Heat: NO actual Heat — extreme Cold forces Yáng upward as last refuge. Warm the patient → false red cheeks disappear.
Tongue rule: Pale WET = Cold always. Red DRY = Heat always. The tongue does not lie.

COLD 寒

- ① Yīn — injures Yáng Qì
- ② Contraction (closes pores → NO sweat)
- ③ Stagnation (pain worse cold)
- ④ Congeals Blood (NOT bleeding)
- ⑤ **CLEAR** discharge
- ✗ **Does NOT cause bleeding**

WIND 風

- ① Yáng — upward & outward
- ② Primary/leader pathogen
- ③ **MIGRATORY** (moving = Wind)
- ④ Rapid changes, abrupt onset
- ⑤ Tremors, convulsions
- Aversion to wind · Sweating (pores open)

HEAT 熱

- ① Yáng — burning & upward
- ② **CAUSES BLEEDING** (key!)
- ③ Consumes Yīn → dryness
- ④ Redness; yellow/thick discharge
- ⑤ Disturbs Shén (irritability)
- ⑥ Stirs Wind (convulsions)

EXTERIOR vs INTERIOR

- EXTERIOR:** Fever + Aversion to Cold TOGETHER
- Floating pulse · Thin white coat · Short/acute
- INTERIOR:** Fever WITHOUT aversion to cold
- OR chills WITHOUT fever (one excludes other)
- Deep pulse · Yellow/black coat · Chronic

DAMP 濕

- ① Yīn — impairs Yáng (Spleen)
- ② Heavy & **TURBID** discharge
- ③ Viscous — prolonged disease
- ④ Obstructs Qì (fullness/nausea)
- ⑤ Settles downward
- Pulse: Slippery · Coat: **STICKY/GREASY**

FULL vs EMPTY — PAIN RULE

- Full pain:** WORSE with pressure · Severe/intense · Strong voice
- Empty pain:** BETTER with pressure + warmth · Dull ache · Weak voice
- Summer Heat:** SUMMER ONLY · No endogenous form

Maciocia 30.1–30.3 Fast Reference

Full Heat vs Empty Heat

Full: whole face red all day · strong thirst (large cold) · yellow coat · Full-Overflowing pulse
Empty: malar flush only · afternoon/eve · dry at night (sips) · PEELED tongue · Floating-Empty-Rapid

Full Cold vs Empty Cold

Full: bright white · sharp pain WORSE pressure · thick white coat · Full-Tight-Deep pulse
Empty: dull white · dull pain BETTER pressure/warmth · thin coat · Weak-Slow-Deep pulse

False Heat / False Cold

True Cold–False Heat: red cheeks like powder · PALE WET tongue · Minute-Deep (hollow) · cold curled body
True Heat–False Cold: cold limbs · RED DRY tongue · Full-Deep pulse · hot chest/abdomen
TONGUE = diagnostic anchor